

Regina Lead Program — Public Brief Bundle

Plain-language summary, investigative timeline, policy-shift scorecard, decision-to-delivery tracker, human-impact health-burden framing, and outward-facing story assets.

Prepared: 2026-03-16 · Evidence standard: primary City of Regina records + conservative JTB documentary validation

Bottom line

Regina did not stand still on lead service connections. Council and committee records show a sequence of formal actions from 2021 to 2025 that expanded replacement rules, financing access, filter support, and reporting direction.

Best current reading: this is **real incremental policy movement**, supported by official records, with **sustained public pressure present in the process**. What the evidence does **not** prove is that any single delegation directly caused a later vote.

Human-impact frame: this is not only a policy-process or infrastructure story. The project's research base also supports a cautious child-development and long-run health framing: lead exposure is linked in the literature to cognitive impacts, ADHD-type burden, and increased later chronic disease risk. That means delay should be judged not only in dollars, but in quality of life and healthy years put at risk.

Supported

Formal continuity from 2021 → 2025

Supported

Delegate / public pressure present in official records

Not proven

Direct causal effect of one delegation on one later decision

Timeline of key decision points

2019-11-25 — MN19-23

Mayor Michael Fougere tables motion to accelerate the Lead Service Connection Management Program.

2021-05-26 — CR21-90

Council approves a 15-year replacement program to 2036, mandatory private-side replacement when City side is replaced, payment supports, financing direction, filter distribution direction, and removes MN19-23 from the outstanding list.

2022-03-09 / 2022-03-16 — EX22-23 → CR22-31

Implementation-facing bylaw changes move from Executive Committee to Council in a short routing cycle.

2023-09-13 — CR23-97

Council continues oversight via recurring update/reporting direction.

2025-09-17 / 2025-09-24 — EX25-82 → CR25-115

Council approves another package of affordability, reimbursement, outreach, and reporting adjustments.

Lag map highlights

Concern / pressure anchor	First mapped response	Lag	JTB note
MN19-23 acceleration motion	CR21-90 program report	548 days	Documentary chain accepted; causality still investigative.
DE21-185 CACA delegation	EX22-23 lead enhancements	287 days	Concern/response documentary presence accepted.
DE21-186 Steve Wolfson delegation	EX22-23 lead enhancements	287 days	Concern/response documentary presence accepted.
DE22-87 Get the Lead Out delegation	CR23-97 2022 update	469 days	Concern/response documentary presence accepted.
EX22-23 executive action	CR22-31 council advance	7 days	Shows short-cycle internal routing is possible.

True policy shift scorecard

Criterion	Result	Score
Timeline acceleration	No formal earlier completion horizon than 2036 found in reviewed chain.	0
Equity execution rule	No clearly codified, auditable high-risk prioritization rule in current set.	0
Delivery standards	Implementation directions exist, but enforceable service-standard language is not clear in reviewed minutes.	0
Transparent performance	Recurring public reporting direction is explicit.	1
Resource lock-in	No explicit accelerated-goal multi-year resource lock found in current set.	0

Total: 1 / 5 → Incremental movement, not a true policy shift yet.

Decision-to-delivery watchlist

Node	What to verify next	Status
CR21-90 baseline package	Track later reports against 2021 directions	Adopted baseline
EX22-23 / CR22-31	Confirm exact bylaw amendment clauses in operative text	Implementation routed
CR23-97	Check whether recurring reporting cadence was met	Oversight continues
CR25-115 rec 2(a)	Verify annual reporting compliance	Future reporting directed
CR25-115 rec 2(c–h)	Verify outreach, financing review, partnerships, and Q4 2026 follow-up	Directed, not yet proven delivered

Health burden beyond dollars

Lead exposure should not be framed only as a financing or engineering issue. The evidence set assembled in this project supports a cautious public-health framing: prolonged exposure is linked in the literature to child-development impacts, behavioural impacts, and increased later-life chronic disease risk.

Burden lane	What current evidence supports	Public use now	Caution
Child cognitive burden	IQ-related developmental harm is supported by the current research set.	Yes	Do not convert to one headline city QALY figure without full utility mapping.
ADHD / behavioural burden	ADHD-type associations are already present in the extracted evidence base.	Yes	Avoid double-counting with broader cognitive-development harms.
Adult chronic disease burden	Cardiovascular and CKD burden are supported as later-life health concerns.	Yes, cautiously	Intermediate-to-final outcome mapping should stay conservative.
Healthy life years	The project can discuss delay in terms of quality of life and healthy years at risk, while keeping any numeric healthy-life-year estimate explicitly provisional.	Yes, as framing	Full numeric QALY/DALY estimates remain a next-step model, not a settled headline.

Conservative public message: the cost of delay should be understood in both dollars and human terms — reduced quality of life, impaired development, and preventable lifetime harm.

Technical backbone: reports/regina_health_burden_qaly_framework_2026-03-16.md, reports/regina_health_burden_endpoint_table_2026-03-16.md, reports/regina_health_burden_public_table_2026-03-16.md, reports/regina_healthy_life_burden_scenarios_2026-03-16.md, reports/regina_burden_weight_literature_review_2026-03-16.md, reports/regina_flint_calibration_memo_2026-03-16.md, reports/regina_child_cognitive_burden_prototype_v2_2026-03-16.md, reports/regina_child_burden_crosswalk_duration_hardening_2026-03-16.md, and reports/regina_health_burden_technical_appendix_2026-03-16.md.

Technical appendix — child cognitive burden envelope

This appendix converts the project's child-development concern into a narrow technical burden estimate. Using a conservative mild developmental-burden crosswalk, a low burden-share assumption, and a medium persistence horizon, the child-cognitive lane alone produces a base-case model output of **3.696 YLD-equivalent units** under the appendix assumptions.

Interpretation: this is a bounded appendix estimate, not a headline public claim. It uses a proxy child exposure-context denominator, excludes ADHD stacking, excludes adult chronic disease

and mortality lanes, and should not be read as a diagnosed-case count or final attributable citywide burden total.

Appendix artifact: reports/regina_health_burden_technical_appendix_2026-03-16.md.

Primary sources

1. City Council Minutes — 2021-05-26 (CR21-90), ID 2818
2. Executive Committee Minutes — 2022-03-09 (EX22-23), ID 2938
3. City Council Minutes — 2023-09-13 (CR23-97), ID 3151
4. Executive Committee Minutes — 2025-09-17 (EX25-82), ID 3440
5. City Council Minutes — 2025-09-24 (CR25-115), ID 3450

This bundle is intentionally conservative. It supports a clear investigative narrative without pretending the record proves more than it actually does.